

HAROTESSA ASSOCIATION-ETHIOPIA (HAE)



Strategic plan

(2024-2028)

December 2023

Hosanna Ethiopia

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ABBREVIATIONS AND ACRONYMS

AIDS	Acute Immuno Deficiency Syndrome
ANC	Antenatal Care
AYSRH	Adolescent Youth Sexual and Reproductive Health
ChSA	Charities and Societies Agency
CSO	Civil Society Organization
FGC	Female Genital Cutting
FP	Family Planning
HDA	Health Development Army
HEW	Health Extension Workers
HEP	Health Extension Program
HIV	Human Immune Virus
HRH	Human Resources for Health
ICT	Information and Communication Technology
KM	Knowledge Management
M&E	Monitoring and Evaluation
MCH	Maternal and Child Health
MDG	Millennium Developmental Goals
MoH	Ministry of Health
NGO	Non-Governmental Organizations
PNC	Postnatal Care
PwD	People With Disabilities
RH	Reproductive Health
SRH	Sexual and Reproductive Health
STI	Sexually Transmitted Diseases
TVET	Technical and Vocational Education and Training

EXECUTIVE SUMMARY

The Harotessa Association – Ethiopia (HAE) has been implementing a range of health and development projects under its core strategic areas of intervention that saved lives, protected health and improved the living conditions of the needy population. The contribution and achievements of HAE are widely known by communities, the government, partners and the general public at large.

The organization, has conducted a review development of its strategy for five years (2024-2028). This strategic plan serves as a strategic guidance on the options that HAE has laid out for the entire planning period. The development of this strategic plan document passed through participatory processes, reviews and consultations with staff and management of HAE as well as other stakeholders, relevant government offices and community based organization and target groups.

Key strategic issues were identified from organizational effectiveness and efficiency and program development, implementation and results areas. The first sets of strategic issues which were seen from the organizational development, efficiency and effectiveness domain have brought core issues that need to be given a strong emphasis for HAE to continue to grow and become effective. The second sets of strategic issues were focused towards the core issues behind the organization's fundamental purpose, existing and persisting problems that have significant impact on the lives, livelihoods, health and wellbeing of the primary targets. Thus, this strategic plan has identified four broader areas for engagement in the five year period. These are Social Inclusion and Gender Equality; Education and Vocational Skill Training; Adolescent, Youth Sexual and Reproductive Health (AYSRH) and Enhancing Livelihood Opportunities for youth and women

HAE recognizes the central role of its members, volunteers, stakeholders, the community and staff in achieving this ambition over the coming five years. Monitoring and evaluation will be carried out based on the indicators developed subsequently. The major assumption in the strategic plan is that the current socio-economic and political situation will remain conducive for the implementation of the objectives and strategies and also the availability of the financial resources estimated to be **5,606,832 (Birr five million six hundred six thousand eight hundred thirty two)**, is expected from the existing and future partnerships.

1. INTRODUCTION

1.1. Brief Overview of the Genesis and Evolution of the Organization

1.1.1. Genesis and Evolution of the Organization

Harotessa Association Ethiopia (HAE) is a non-profit, non-governmental organization established by a committed members of the marginalized Gafat Beta Israel Community (GBIC) in 2010. The main objectives of HAE are combating systemic marginalization and social exclusion of the community; promoting educational opportunities for children of the social caste groups; contributing towards the poverty reduction efforts among the GBIC; raising awareness of the GBIC towards the benefits of access to basic services like education, health (family planning, personal and family hygiene and sanitation) and economic empowerment. Ensuring right of the community in terms of land ownership and participation in social and political activities also lies at the heart of the mission of the organization.

The organization has been contributing a lot by alleviating socio-economic problems of destitute mothers, children and youth, expansion of education and health care facilities. Particularly the organization has been doing tremendous contributions on children and adults at risk since its establishment. With its head office based at Hossana town, HAE is currently operating in Hadiya zone at Sorro woreda targeting 10 kebeles and providing awareness raising and training on primary health, non-formal education and capacity building to self-help groups (SHGs). Furthermore, through a pre-primary (Kindergarten) education service located in its head office compound, enrolled children from GBIC living in Hossana town who were not able to get education because of discrimination and social marginalization. Some committed mothers and adolescent girls were also engaged in potter production and making some earning. HAE is promoting social inclusion and gender equality with a continued interaction and advocacy works.

1.1.2. Evolution of the Organization

By ways of realizing its vision and accomplishing mission, in the last ten years, HAE has been aggressively implementing a range of projects geared towards saving lives, ensuring health and improving the living conditions of the community. Building on experience and lessons from past interventions, HAE diversified its program portfolio to including similar projects focused on improving the life of the community. HAE initiated different projects in various programmatic areas: education, health, livelihoods, and advocacy. That means, in the past 10 years, HAE has gained

significant experience in initiating and implementing development projects of the different types. In doing so, the organization developed solid and commendable experience which proved reliable reputations in development sector.

Table 1: Program Components

S/No.	Name of the Project/Program	Objective/Goal	Sector/sub-sector
1	Fostering/Improving the social-economic level through comprehensive and inclusive Actions(ISEL-CIA) among the socially Marginalized Community	Strengthening local government engagement in ant- marginalization situation in Gafat communities with in community dialogue by organizing SHGs,and building their capacity .	Health, ,Finance, woman, children affair office, Agriculture
2	Improving the social-economic level through comprehensive and inclusive Actions(ISEL-CIA) among the socially Marginalized (GBI-C)Community	To contribute to inclusive and equitable social and economic development for marginalized Gafat communities through evidence based promotion and dialogue with Government &communities.	Health, ,Finance, woman, children affair office, Agriculture
3	Improving the social-economic level through comprehensive and inclusive Actions(ISEL-CIA) among the socially Marginalized Gafat Beta Israel Community(GBI-C)	To contribute to inclusive and equitable social and economic development for marginalized Gafat communities through evidence based promotion and dialogue with Government &communities.	Health, ,Finance, woman, children affair office, Agriculture
4	Promoting Social Inclusion of Marginalized community	Improved access to services for hard to reach people/Gafat in the proposed strategic intervention areas.	Health, ,Finance, woman, children affair office, Agriculture
5	Promoting Social Inclusion and gender equality for the empowerment of Marginalized Gafat Bete Israel Communities and the ultra-poor section of the community	To contribute to the gender equality, social and economic inclusion of marginalized GBI community and the ultra-poor section of the communities in local development initiatives.	Health, ,Finance, woman, children affair office, Agriculture

2. COUNTRY CONTEXT AND ANALYSIS OF THE EXTERNAL ENVIRONMENT

2.1. Country Context

With its total area of 1,127,127 sq.m, Ethiopia is considered as the 27th largest country in the world. The government of Ethiopia is structured in the form of a federal parliamentary republic, where executive power is exercised by the government while legislative power is vested in the Parliament. The Federal Republic comprises of the Federal Government, the 11 autonomous Regional States (*Killils*) and two city administrations. Both the federal and regional governments are organized into three branches of government – legislative, executive, and judicial. The Federal legislative organ constitutes two houses (a bicameral parliamentary system): the House of People's Representatives (HPR) usually known as Lower chamber and the House of Federation (HoF), Upper chamber. The highest legislative authority is vested in the HPR whose members are elected by a plurality of votes cast in general elections every five years. The State Council is the highest organ of the state legislative authority.¹

Ethiopia is the 2nd most populous country in Africa and ranked 13th in the world. The projected population for the year January 1, 2028 (period where this SP ends) was estimated to be 115,946,201 (58,074,006 male and 57,872,195 female). Urban population constitutes 29,311,431 (14,566,208 male and 14,745,223 female) which is 25.3% of the total population. On the other hand, rural population of the country was estimated to be 86,634,770 (43,507,798 male and 43,126,972 female) which is 74.7% of the country's total population.²



Figure 1: Map of Ethiopia

¹ Haradhan Kumar Mohajan. Ethiopia: A socio-economic study. Journal of Business Management and Administration Vol. 1(5), pp.59-74, November, 2013 <http://www.peakjournals.org/sub-journals-JBMA.html> ISSN: 2329-2954 ©2013 Peak Journals.

² FDRE. Central Statistical Agency. Population Projections for Ethiopia (2007-2037). Addis Ababa. July 2013

2.2. Situational Analysis and Review of the External Environment

2.2.1. Economic Growth and Challenges

Ethiopia made significant and continuous economic growth over the past decade. Achievement has also been witnessed in its commitments in the Millennium Developmental Goals (MDGs) completed in 2015. Ethiopia has seen the second fastest rate of reduction of poverty in Africa over the past 10 years with poverty headcount falling from 38.7 percent in 2005 to 29.6 percent in 2011³. However, despite the faster and high economic growth and gains over the past over the past ten years, sustaining the growth was remained a serious challenge. The major challenges were attributed to a number of factors.⁴

One is failure explained by gaps in the areas of creating adequate job opportunities, ensuring equitable distributions, ensuring structural transformations, and creating sectorial linkages and synergies. Secondly, external debt distress and limited competitiveness in international markets constrained sustainability of economic growth. Imbalance between domestic savings and investment characterized by the inability to satisfy the domestic investment demand was the third factor for limiting sustainability. Besides, challenges related to stabilize inflation; rise in unemployment; sluggish structural transformation and weak sectorial linkages; poor capacity to mobilize domestic resources; limited accessibility of financial institutions; deficient and low quality provisions of social services and basic infrastructure were main bottlenecks. Above all, lack of robust system to correct the failures and ensure accountability become additional challenge.⁵

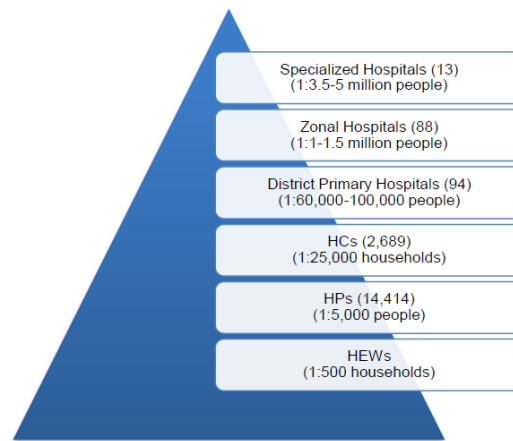
2.2.2. Ethiopian Health System: General Overview

Ethiopia has a three-tiered system of primary, secondary, and tertiary care. Tertiary care is delivery by specialized hospitals with catchment areas of 3.5–5 million people; secondary care is delivered by general hospitals with catchment areas of 1–1.5 million people, and primary care is delivered by primary hospitals, health centers, and health posts serving 60,000–100,000 or more people at the woreda level. The woreda-level primary health care system typically includes a woreda health office, a primary hospital, at least 5 health centers, 25 health posts, and 50 health extension workers (HEW), two per health post.

³World Bank (2015), 4th **Ethiopia Economic Update: Overcoming Constraints in the manufacturing Sector**.

⁴FDRE, Planning and Development Commission. Ten Years' Development Plan. A Pathway to Prosperity. 2021-2030.

⁵ Ibid



Source: FMOH, HSDP IV 2010-2011, 2014-2015, October 2010

Figure 2: Structure of the Ethiopian Health Care System

As more than 80 percent of the Ethiopian population resides in the rural part of the country, more focus is given to the primary health care units while strengthening the referral care at secondary and tertiary levels. Ethiopia's flagship Health Extension Program (HEP)⁶ is the platform for delivery of community based basic promotive, preventive and curative health services. Two Health Extension Workers (HEWs), based in the health posts serve an average of 5,000 populations, and are the primary point of entry to the health system for the rural community. HEWs, supported by the Health Development Army (HDAs) created between five households and one model family to influence one another in practicing healthy life style, work to empower the community to generate its own health⁷.

Ethiopia is addressing major challenges in the health sector, including lack of human resources for health (HRH) and low utilization of health services, through its innovative Health Extension Program (HEP), accelerated midwifery training, Integrated Emergency Surgery and Obstetrics (IESO) task shifting, and scaling up family planning (FP). HEWs were trained to deliver a basic package of preventive and a few curative health services, including maternal and child health services. Thus, over 38,000 health extension workers were trained and deployed under the HEP scheme. In addition, approximately 16,000 health posts and 3,000 health centers have been constructed to increase access to essential services⁸.

⁶ The government has created and deployed more than 38,000 health extension workers, established more than 16,000 health posts and approximately 3,500 health centers, and is now establishing primary hospitals at the district (woreda) level.

⁷ National Newborn and Child Survival Strategy Document. Brief Summary. 2015/16-2019/20. June 2015. FMOH https://www.unicef.org/ethiopia/Child_Survival_Strategy.pdf

⁸ Success Factors for Women's and Children's Health. MoH Ethiopia. WHO 2015: http://www.who.int/pmnch/knowledge/publications/ethiopia_country_report.pdf

Maternal Health

Ethiopia has been ranked fifth in the world as having the greatest potential to save maternal, perinatal and neonatal lives by 2025. The country has achieved impressive progress on child survival, cutting under-five mortality by 67 per cent since 1990 and meeting its Millennium Development Goal 4 target three years ahead of deadline⁹ and is one of only twelve low income countries to do this¹⁰. There has been a steady decline in infant, child, and under-5 mortality over the last 16 years in Ethiopia. For example, **under-5 mortality rates** (<5MR) declined from 166 deaths per 1,000 live births to 123 deaths per 1,000 live births in 2005, to 67 deaths per 1,000 live births in 2016. Similarly, **infant mortality** (IMR) decreased from 97 deaths per 1,000 live births, to 77 deaths per 1,000 live births, and to 48 deaths per 1,000 live births in the same period. The **neonatal mortality rate** (NMR) has declined, in the same period, from 47 deaths per 1,000 live births, to 46/1000 live births, and to 29/1,000 in 2016.¹¹

The leading causes of maternal death in Ethiopia are postpartum hemorrhage (PPH), unsafe abortion, infection, pregnancy related hypertension, and obstructed labor (Koblinsky; et al. 2010). Ensuring availability of modern contraceptives and essential life-saving maternal and reproductive health care is a major challenge in Ethiopia, where unmet need is very high and funding for supplies is almost completely donor dependent (Morrison and Brundage. 2012)¹². Moreover, 80 percent of births to urban mothers in Ethiopia were assisted by a skilled provider and 79 percent were delivered in a health facility, as compared with 21 percent and 20 percent, respectively, of births to rural women. Mothers' educational status is highly correlated with whether their deliveries are assisted by a skilled provider and whether they are delivered in a health facility.¹³

As regards to the critical MCH indicators such as ANC, PNC, institutional delivery, etc. results from the 2019 mini-DHS indicted a steady improvement. The report showed that 74 percent of women age 15-49 with a live birth in the 5 years before the survey received antenatal care (ANC) from a skilled provider for their most recent birth. Urban –rural disparity is high in terms of use of ANC. In this

⁹ Levels and Trends in Child Mortality. Report 2015. Estimates Developed by the UN Inter-Agency Group for Child Mortality Estimation. UNICEF. <http://www.apromiserenewed.org/focussing-on-every-newborn-ethiopia-taking-action/>

¹⁰ Including Cambodia, Ethiopia, Eritrea, Liberia, Madagascar, Malawi, Mozambique, Nepal, Niger, Rwanda, Uganda, and United Republic of Tanzania.

¹¹ Ethiopia Demographic and Health Survey. Key Indicators Report 2016. Central Statistical Agency, Addis Ababa, Ethiopia. October 2016.

¹² Raifman, S. et al. *Assessment of the availability and use of maternal health supplies in the primary health care system in Amhara region, Ethiopia*. Final Report. Population Council 2013

https://www.popcouncil.org/uploads/pdfs/2013RH_MaternalHealthSuppliesEthiopia.pdf

¹³ Ethiopia Demographic and Health Survey. Key Indicators Report 2016. Central Statistical Agency, Addis Ababa, Ethiopia. October 2016.

case, urban women (85%) are more likely than rural women (70%) to receive ANC from a skilled provider. Furthermore, education is an important factor to get ANC services from a skilled provider. That is, ANC utilization increases with increasing mother's education, from 62% among women with no education to nearly 100% among women with more than a secondary education. Women in the highest wealth quintile (95%) are more likely than those in the lowest quintile (47%) to receive ANC from a skilled provider. The report further disclosed that institutional deliveries increased from 26% in 2016 to 48% in 2019, while home deliveries decreased from 73% to 51% over the same period. On the other hand, 34 percent of women and 35 percent of newborns received a postnatal check within the first 2 days after birth which showed low performance.¹⁴

Child Health

The 2019 mini DHS results show that the neonatal, infant, and under-5 mortality rates for the 5 years before the survey were 33, 47, and 59 deaths per 1,000 live births, respectively. In other words, 1 in every 30 children in Ethiopia die within the first month, 1 in every 21 die before their first birthday, and 1 in every 17 die before their fifth birthday. Rural urban disparity is clearly observed in under 5 mortality. Mortality rate of the under 5 infant is higher in rural areas than in urban areas (51 and 32 deaths per 1,000 live births respectively). The under-5 mortality rate decreases with increasing household wealth. Under-5 mortality is 77 deaths per 1,000 live births in the poorest households and 46 deaths per 1,000 live births in the wealthiest households¹⁵. That is, among others, settlement (rural) and poverty explained by a low household income contributes towards increased under 5 infant mortality rate.

Sexual and Reproductive Health (SRH) Environment

SRH being a comprehensive concept and implies a broad range of health and non-health related interventions, overview of the larger social and institutional contexts that influence reproductive health (RH) in Ethiopia. It highlights the ways in which those conditions affect the RH status of women and men in the country. To this end, poverty, education, the legal environment, status of women and the health care system become crucial aspects. Poverty is considered as one of the most important determinants influencing the RH status. Ethiopia is one of the poorest countries in the

¹⁴ Federal Ministry of Health. Mini Demographic and Health Survey 2019;. Ethiopian Public Health Institute. Addis Ababa. May 2021

¹⁵ Ibid

world, with an estimated gross national product (GNP) per capita income of US\$100. Today, it is estimated that 47 percent of the population still lives below the poverty line¹⁶.

On the other hand, the low status of women in Ethiopia underpins, and often directly undermines each of the negative RH outcomes addressed in the national strategy. Most Ethiopian women lack the reproductive and social self-determination needed to exercise their reproductive rights – a condition that, in turn, perpetuates their low RH and social status. Despite a biological tendency towards higher survival rates, girl children are 4 percent more likely to die during infancy than boys. Between 60 and 80 percent of Ethiopian women have experienced some form of female genital cutting (FGC), a practice with potentially severe health consequences.¹⁷

The health care system, traditionally the public sector which was overburdened and has primary responsibility for the delivery of health care services through government-owned facilities. Nevertheless, since recent years, non-governmental organizations, private for-profit clinics, traditional practitioners and rural drug vendors together make up about one third of all service provision. RH is not a standalone program in the health care delivery system. Instead, it is integrated and mainstreamed across the health sector. That means, facilities are shared, staff are shared, resources are shared, and opportunities (both realized and lost) are shared.¹⁸

Diversity is another factor worth considering in relation to SRH. Ethiopia is characterized by unique demographic, cultural and geographic diversity. It entails for multiple and diversified approach that responds to a number of social and cultural elements. Apparently, the need arises to understanding factors that effectively differentiate society and their RH needs. In this case it is vital to account for inclusion-exclusion aspects considering the pastoral populations, urban migrants, aging population, adolescents, displaced populations, those in conflict situations, etc. In other words, it means using approaches that are needs-driven and segmented by residence, geography, culture, age, marital status, disability status, sex, etc.¹⁹

¹⁶ Federal Democratic Republic of Ethiopia MINISTRY OF HEALTH NATIONAL REPRODUCTIVE HEALTH STRATEGY 2006 - 2015 March 2006

¹⁷ Federal Democratic Republic of Ethiopia MINISTRY OF HEALTH NATIONAL REPRODUCTIVE HEALTH STRATEGY 2006 - 2015 March 2006

¹⁸ Ibid

¹⁹ Ibid

RH of young people – reports indicated that while approximately 60 percent of Ethiopia’s population falls between the ages of 10-29, few national programs or policies are specifically targeted towards addressing their most pressing RH needs. In urban areas, new research suggests that existing RH coverage is limited, with only 12 percent of young people sampled in Addis Ababa visiting youth centers and only 20 percent reached by peer educators. In addition, most programs for young people in Ethiopia, as well as in sub-Saharan Africa generally, tend to deliver generic, age- and gender-blind messages that fail to recognize the distinct needs of girls versus boys at different ages, as well as the unique needs of married adolescent girls. This limited access to targeted RH care and services for young people contributes to, and exacerbates, many of the RH problems outlined above.²⁰

²⁰ Ibid

3. EXPLANATION OF THE PROCESS, METHODOLOGY USED AND ACTIVITIES COMPLETED

The following steps were followed as a basic approach and processes in the course of developing the strategic plan. Major steps included initiation consultations; general discussions; document review; interviews and analysis of strengths, weaknesses, opportunities and threats; and assessment of national policies and strategies.

3.1. Initiation consultations

The initiation consultations were conducted with the key staff from program department and management teams. Main agenda of the consultation was deliberations on the next five years strategic directions as an entry point of the overall strategic planning process.

3.2. General Discussions

General discussion was conducted with the management of the organization led by the Executive Director, management members and others. Management members were engaged in to the main subject matter focusing on the vision, mission and values of the organization. Management members discussed over whether the existing vision, mission and values could be revisited. The management eventually agreed that these components being fundamental elements of the organization need to be revisited.

3.3. Document Review

Pertinent documents like previous strategic plan document; project proposals; reports; human resource manuals; and financial management manuals were gathered from the organization and carefully reviewed.

3.4. Interviews and Group Discussions

Interview with different persons was conducted. Internally, interview was conducted with program staff and HAE board members. Externally, experts and officials from government organizations; different institutions; representatives from other agencies were interviewed.

3.5. SWOT Analysis conducted

Organization's strengths, weaknesses, opportunities and threats were assessed during groups' discussions with HAE staff. Inputs were also gathered from external stakeholders.

4. ANALYSIS OF THE ORGANIZATIONAL CONTEXT

4.1. The Journey of in the Past

Since its establishment (2010), HAE developed and supported programs in Hadiyya zone, SNNPRS involving income generation, self-help savings and credit groups, vocational and literacy training, children's education, health services, preventative health education, etc. Hence, HAE sets an ambitious goal that aspires to use a holistic support and empowerment approach to enhance the livelihood of the target groups; those leading destitute life due to marginalization, social exclusion and extreme poverty, and to provide the target group opportunity to strengthen and preserve their family and to become economically self-reliant. Working in partnership with the government and Ethiopian local NGO for the past years, has HAE now broadened its reach to strengthen its institutional and technical capacity. In terms of track record, the following are some of the projects implemented and successfully delivered.

- Fostering/Improving the social-economic level through comprehensive and inclusive Actions(ISEL-CIA) among the socially Marginalized Community
- Improving the social-economic level through comprehensive and inclusive Actions(ISEL-CIA) among the socially Marginalized (GBI-C)Community
- Improving the social-economic level through comprehensive and inclusive Actions(ISEL-CIA) among the socially Marginalized Gafat Beta Israel Community(GBI-C)
- Promoting social inclusion of marginalized Gafat Beta Israel communities

4.2. Analysis of Strengths and Weaknesses

Results of interview and discussions on the organization's strengths and weaknesses provided some essential outcomes worth considering in the future. Details are provided below in Table 2.

Table 2: Internal Strengths and Weaknesses

Strengths	Weaknesses
Governance, Board, Leadership and Legal Status	
• strong membership base – support and participation of the different segments of the targeted community to uphold organization’s objectives high • aspiration to increased geographic coverage, increased outreach to the community in need • provision of children’s education by running a primary school and support from local government • represented the marginalized community and effort to advocate their voice • endeavor to diversify partnership and program, • engaging in advocacy activities to reach population at risk • basic working materials and strategic policies • availability of motivated staff with passion to strengthen the association • meeting compliance requirements internally and externally • developing governance system building able and matured leadership • Board members meets periodically to discuss on progress and approve plans; and have a good practice in monitoring and reviewing organizational function and constituency	• slow pace in selling organization’s best reputations and lack of a well-developed electronic/digital media system • slow pace of networking and dissemination to increase public visibility of the organization • last SP was not evaluated and reviewed • Lack of organized membership data disaggregated by geographic, gender and other socio demographic characteristics of members • Board members’ effort in increasing profile of stakeholders and external relationship and resource mobilization is limited; engagement of board in strategic partnership; representation of board with partners • lack of clear roles and responsibilities among association’s members and staff
Organizational Vision, Mission, Strategies and Work planning	
• There is a written vision and mission statement for the organization • Mission and vision, being used in strategic communications, posted in key locations such as project, regional and head offices • The organization has prepared and implemented a five-year strategic plan • The strategic plan is used as the basis of the organization’s annual plans, and it appears to be used regularly as a leadership tool	• Mission and vision are ambitious and lacks specificity as to the organization’s comparative advantage and specific offers • Inconsistency in using and communicating the organization’s vision • Lack of objectively measurable and specific targets in strategic and annual plans
M&E, Knowledge Management, Communication and Organizational Sustainability	
• Has fairly sufficient monitoring and evaluation practice for projects implemented by the organization	• Systems for consistently capturing, documenting and sharing knowledge in the organization are weaker Collecting and

• Conducts joint monitoring and review sessions with partners • Have worked with diversified groups of donors • Received positive feedbacks and appreciation from donors and governmental partners • Have built strong relationships and partnerships with stakeholders at the implementation level	documenting success stories and lessons learned not done consistently to ensure strong knowledge management system • Limited efforts in fundraising and inadequately set or track proposal submission targets or win rates • The organization does not have a website that is continuously being updated with relevant contents
Organizational Structure and Management	
• There exists an organizational structure that puts most of important functions in the organization	• Organizational structure doesn't include some essential structures and functions that can be further strengthened through time • Lack of clear reporting and accountability lines and linkages to key organizational functions
Human Resources Development and Management	
• There are travel policies and procedures in place • There exists basic and functional HR manual • Reference checks are being conducted during new staff recruitment • Gender composition in staffing, both at head and field levels is encouraging • Conducts exit interview when staff leave the organization	• There is no up-to-date staff retention strategy in place • There is no clear performance management and career advancement strategy or plan • Inadequate organization and documentation of personnel files • Limited opportunities for staff development • Insufficient staff motivational schemes • lack of comprehensive benefit (insurance; transport; medical) package; pay structure/system; staff retreat
Financial Systems, Budgeting and Property Management and Operational Policies	
• Budget utilization reports are submitted to donors on a monthly or quarterly basis, depending on the grant agreements. • Have basic financial policies and procedures manual, good experience in donor compliance and reporting • There is a segregation of duties for requesting, approval of requests, preparation of vouchers, approval of cheque and vouchers • basic financial control mechanism including reconciliation with field office	• Master organizational budget is not clearly linked to the strategic plan and annual budget plan • There is no multi-year forecast for revenue and expenditures • No updated property management guideline

done on a monthly basis • There was no major incident identified through audits internally or externally • Clear and organized documentation of accounts and financial reports • Transparent and strong policies and procedures in procurement and fixed asset management	
Working Space and Office Facilities	
• office premise built in the land secured from government and was strong achievement • Have fairly sufficient working spaces in offices	• Inadequate ICT infrastructure – data base; networking; digitalizing; SharePoint; attractiveness of the office environment
Technical and Program Operations	
• The organization is developing experience in program development and management • Targets the neediest and usually neglected groups and most at risk population • Uses government, religious and community structures to deliver programs and services ; Good in crosscutting issues of gender, environment and disability in to the key project activities • Good in collecting baseline information through gap assessments to inform actions	• Operation is limited in selected geographic areas. • Sometime there appears a lack of clarity on organizational focus and primary targets • Comparative advantage in pioneering and leading was not sufficiently leveraged (on advocating marginalization and exclusion); • Have limitations in building a stronger technical capability on the key focuses of the organization

4.3. Analysis of Opportunities and Threats

Table 3: Opportunities and Threats

Areas/ Factors	Opportunities	Threats
Technical	• visibility by donors, partners and government; Availability of professional expertise within HAE • Availability of committed potential volunteers across intervention weredas and zones to promote the work of HAE; volunteers as needed; HAE is trusted by donors as well as partners	Fear of decline in funding. Unless and otherwise the organization treats this strategically and timely, financial resources may be declined at some point in time; and hence survival of the association may be at risk
Political	• Existence of favorable government policies and good political commitment on education, health, HIV/AIDS, SRH, Climate Change and other development issues • Regional and local government structures provide necessary supports when there is a need • secured land from local government • pottery work may develop in to a big business enterprise • Sector departments and offices in Hadiyya zone keen to support the association	• Gaps in implementing pertinent policies and strategies by the respective government authorities • Some stakeholders may not be as supportive as they are expected to be; Instability and insecurity that could affect program implementation • low team spirit and weak group cohesion among staff and members
Economic	• engage in strong business development activities; Improving health and education infrastructure at the community level • High commitment from the government to promote microfinance and enterprise development interventions in low income communities • Access to credit services has improved –an ample opportunity for women's' economic empowerment	• Limited economic and employment opportunities for youth • High inflation rate and increasing living cost • Lack of land and other essential infrastructures such as water, power and market outlets for engaging women and youth in income generating activities (IGAs) • Limited livelihood and employment opportunities for marginalized communities residing in different areas • Donor fatigue and shrinking funding opportunities
Social	• Improved educational opportunities that will contribute to better outcomes in health, economic wellbeing • Improving health care seeking behavior of the community • High interest from the community on the project activities that the organization is currently implementing on –puts the collaboration and participation at a high caliber	• Increasing number of vulnerable population and risky behaviors • Un-informed rural urban migration and trafficking of persons • Lack of youth friendly services and service providers • man-made and natural disasters such as eruption of civil war/conflict; displacement; prolonged drought and floods

Technological	• Rapid changes in technology and introduction of new tools and techniques that can transform work • Access to information communication technology is becoming less difficult; Increased access for delivery of online services; hence reduced cost of business	• High cost of acquisition and maintenance of the ICT infrastructure due to rapid technological changes • Weak ICT establishment and readiness to implement changes in the near future; ICT literacy and experience limited in the youth population in rural areas
Legal	• The latest ChSA legislation put a clearer guidance, supporting and monitoring mechanisms that ensures transparency and accountability; Charities and societies legislation allows the CSO's to engage in public collection and income generating activities • The federal and regional governments have clearer guidance in administering and regulating the CSOs	
Environmental	• Ethiopia key player in global climate change leadership and the green growth	• Global warming and environmental degradation • Rapid increases in natural disasters, drought and global warming • New health hazards and increasing spatial and temporal distribution of communicable diseases due to climate change/weather variability

4.4. Conclusion on SWOT Analysis

HAE demonstrates internal organizational strengths that can be used as a starting point for its effort in achieving its results and bring about organizational change and shaping the organization's future. Continuous commitment and dedication to transform the organization is seen from the board, staff and management, which is a massive asset. Board's commitment is demonstrated by frequent meetings to discuss on organizational issues, approving plans and reviewing progress. However, commitment of board in areas such as in fund raising has been limited. The board also shows openness to contribute more when needed. Organizational systems and procedures are backed up with written standard policy and procedure manuals that are being applied by the staff and management in finance, human resources and other administrative issues which are essential in promoting transparency and accountability within the organization.

The organization has a strong and positive relationship with its partners: government; non-government; community organizations; beneficiaries and target groups. The credibility and reputation gained can be a vital and instrumental to influence future collaboration and partnership building efforts that HAE has outlined to achieve in the next 5 years (2024-2028). HAE complies with government and donor requirements and has no any issue of concern in the near future and the organization will stay stable. The organization's deep experience on the main issues embedded into the current and upcoming programs will be of an added value to its effectiveness in the future, and also built trust to stakeholders working with it. In order to successfully implement the strategic plan and pave ground for achieving the aspired vision, the gaps that the organization identified needs to be addressed.

The key areas spotted for weakness during the internal environmental analysis includes inadequate involvement of the board in resource mobilization, lack of strong and continued action in fund raising and partnership, absence of staff development opportunities, absence of vibrant system in the HR, M&E and knowledge management (KM) and limited visibility at national and international levels on issues that it has a very strong presence and reputations. The strengths mapped within the organization will be essential in overcoming the threats identified in the external environment and minimize the impact in program operation and organizational sustainability and also address internal weaknesses, also leveraging from the opportunities expected from the external environment as listed in the table above.

As stated above, the board and staff commitment towards the organization's purpose is remarkable. Further to this, these groups represent the most valuable assets for the organization, on which successes are relied on. Hence, having a committed, talented and inspired staff, board and management can add a

power for HAE and this power can be used to minimize the impact of the external threats and reduce internal weaknesses. However, there appears a need for a continuous discussion and review on the system in order to improve its approaches on human resources and leadership, in order to strengthen the motivational and inspirational factors and reduce issues that could possibly play against this.

Although organizational systems are fairly sufficient to ensure accountability and a transparent process across the budget management, recruitment and administration functions, there appears a need to make continuous improvements to deal with organizational growth and expansion and in order to ensure compliance and meeting requirements and needs by the donors, supporters and the government. Strategic communication needs to be enhanced and used as a means to engage more supporters and donors through disseminating relevant information about what the organization has done and results gained, including its intentions in the future.

5. CONSULTATIONS WITH THE STAKEHOLDERS

5.1. External Stakeholders

As part of the strategic revision and planning process, HAE has conducted a series of consultations with stakeholders in order to collect and reflect their views about the organization's work in to the strategic plan. The consultation with these stakeholders has also informed us in developing and validating strategic focuses for our strategic plan.

5.2. Internal Stakeholders

Being the most valuable assets for the organization, internal stakeholders (staff, management and board members) play key role in implementing its strategies both in the short and long terms. Staff and management members within the organization, who are working in fulltime, were represented in the strategic planning team –providing supports and overall guidance to the consultant. The board members, being the high level decision makers under the general assembly, have conducted meetings with the planning team and provided feedbacks and reviews throughout the process.

The staff, management and board members well understood the need for developing a new strategic plan for the period from 2024-2028, thoroughly reviewing the current situations both within and outside the organization. The need for strategic actions on key and persistent issues that are affecting the organization was also highly reflected.

5.3. Stakeholders Analysis

The organization highly recognizes the central role of stakeholders in achieving its vision. Hence, the process of developing this strategic plan significantly involved stakeholders at different levels in the identification of the strategic priorities and provided feedbacks and accommodated their reflections on their expectations. This has ensured the stakeholders consciously accept and commit for supports.

5.4. Feedbacks from Stakeholders

Selected stakeholders consulted during the strategic planning process provided feedbacks to the organization based on their participation in the implementation of the programs funded by them. Stakeholders have shared their perspectives, feedbacks and an assessment of challenges and opportunities in the external environment as well as recommendations for the organization. The key feedbacks provided from the internal stakeholders have been used in the development of the strategic objectives and overarching issues reflected in the strategic plan.

5.5. Internal gaps that needs to be filled

Stakeholders critically reflected on the accountability mechanisms embedded in to resource consumption and allocation witnessing that HAE allocates significant proportion of money in the benefit of the target community. The responsiveness by the leadership, the very strong bond with the community and local stakeholders including wide acceptability and volunteerism, community mobilization experiences and capacities, mobilizing community contributions for projects and interventions are the areas that stakeholders believed the organization has demonstrated strengths. However, it was also denoted that the organization needs to put more efforts towards improving the technical capabilities, strengthen program quality control and involvement and participation in taskforces and networks at different levels.

5.6. Challenges Faced

Stakeholders also stressed the existing challenges for organizations like HAE in staying along their strategic pathways, experiences and the communities' need as donors are the biggest influence in shaping the directions for programs; and even organizational visions and directions.

Most donor funds are coming with several issues already defined (tailor made), there is a small room for NGOs to influence. Lack of harmony and coordinated approach in the CSO sector; poor practices of hasty generalization like labeling some CSOs by authorities, donors and the public; gaps in the knowledge and understanding on development practices; lack of strategic flexibility and adaptation in the changing environment and the limited opportunities in CSO development and capacity building supports by donors are also among the perceived bottle necks.

Summary of Feedback from Internal Stakeholders

- Consider a revision on the current vision, mission, thematic areas and primary targets
- Consult external stakeholders during the strategic planning process
- Scale up existing interventions, incorporating lessons and innovations
- Intensify efforts to diversify donor/revenue base and widen the financial resource bases
- Improve systems and working environment for staff
- Increase staff development, retention and career advancement opportunities
- Strengthen partnerships and networking at community, regional, national and international levels
- Expand programmatic focuses beyond existing interventions by making them responsive to the emerging needs of the primary target groups
- Revise organizational systems, policies, structures and functions in line with the fast-changing circumstances (change management)
- strengthen team spirit and group cohesion among staff and members of the association
- ensure high adherence to association's rules and regulations and that of the rule of the land

Figure 3: Summary of feedback from Internal Stakeholders

5.7. Recommendations, Feedbacks and offers from External Stakeholders

Recommendations and Feedback from External Stakeholders

- Continuous learning and modification of projects and programs
- Improved coordination and harmonization within the sector and engage in constructive dialogue with the government and donors
- Strengthened internal organizational communication and partnership framework
- Enhance reputation and credibility by the government, donors, community and other partners
- Elevate efforts in strategically communicating, presenting and promoting
- Explore and expand membership in the existing national and regional coordinating platforms and networks
- Look for local income-generating opportunities and schemes; and expand the existing and on-going pottery production schemes to modern system that attract market in innovative way
- Critically review current thematic issues and strategic pillars in order to make sure that the organization's interventions are well aligned to the development challenges at the community level and also connected to national frameworks and the strategic direction of the organization
- Follow youth-friendly, holistic and integrated approaches in programming
- Strengthen partnerships with youth groups and other CBOs in conducting community mobilization and training activities
- Increase geographic scope –beyond the current target areas, but focus on selected thematic issues on which the organization has already gained strong experience on
- Create and improve linkages with different stakeholders at different levels –including referral linkages
- Strengthen strategies for phase-out and ensure smooth handover of project activities to the designated government and community structures
- Strive to become a ' of excellence' on social inclusion and addressing marginalization related issues in Ethiopia
- Increase youth and women-focused health and livelihoods interventions
- See the 'bigger picture in partnerships' not only the shorter gains
- Mobilize more volunteers and members

Figure 4: Recommendations and Feedbacks from External Stakeholders

Offers from External Stakeholders

- Share experiences in income generating projects to HAE
- Participate and contribute in joint planning and review platforms
- Create and strengthen referral linkages for services to beneficiaries
- Provide technical support through resource persons and professionals
- Willingness to provide land for clay soil use
- Willingness to use affirmative action if candidates from the GBIC qualify for employment in the public service department or elsewhere in Hadiyya zone

Figure 5: Offers from the External Stakeholders

6. HAE STRATEGIC PLAN (2024-2028)

6.1. Purpose of the Strategic Plan

HAE prepared this strategic plan with the overall purpose of adopting a strategic guidance on the efforts towards achieving its vision through shaping its fundamental purpose and actions required.

6.2. Vision, Mission and Values

6.2.1. Vision

- To see a free Gafat Bete Israel Community from inherent exclusion

6.2.2. Mission

- HAE is committed to reduce the deep and abject poverty where women and disadvantaged communities are free from all forms of discrimination and violence and are able to attain equity and equality for themselves and societies at large.

6.2.3. Organizational Values and Principles

- Equality
- Participation
- Volunteerism
- Dignity
- Integrity
- Transparency
- Accountability
- Partnership

6.3. Analysis and Results: Strategic Issues

6.3.1. Social Inclusion and Gender Equality

Sources indicated that social exclusion means a situation where not everyone has equal access to the opportunities and services that allow them to lead a decent, happy life. This in other words means that not being able to give input and have their voice heard on the rules of the society in which they live. Such people are denied the right to access to and opportunities and services like infrastructure; electricity; running water; public education, healthcare or the social welfare system.²¹ Other literatures in social exclusion also include exclusion from livelihood opportunities; secure & permanent employment; earnings; property, credit, or land; housing; minimal or prevailing consumption levels; education, skills, and cultural capital; the welfare state; citizenship and legal equality; democratic participation; public goods; the nation or the dominant race; family and sociability; humanity, respect, fulfillment and understanding.²²

²¹ <https://www.liberties.eu/en/stories/social-exclusion/43579> (Retrieved as of May 24, 2023)

²² Amartya Sen. SOCIAL EXCLUSION: CONCEPT, APPLICATION, AND SCRUTINY. Master of Trinity College, Cambridge, and Lamont University Professor Emeritus, Harvard University Social Development Papers No. 1 Office of Environment and Social Development Asian Development Bank June 2000

Some authors also argue that social exclusion have multidimensionality. According to them social exclusion is multidimensional encompassing broad issues like a lack of access to employment, legal redress and markets; a lack of political voice; and poor social relationships. Authors therefore argue that it is not enough to examine these issues individually, and that the links between them must be explored.²³ Extending the multidimensional nature of social exclusion, still argued that social exclusion is a complex and multi-dimensional process. It involves the lack or denial of resources, rights, goods and services, and the inability to participate in the normal relationships and activities, available to the majority of people in a society, whether in economic, social, cultural or political arenas. Social exclusion affects individuals, groups and communities in terms of the quality of life of individuals and the equity and cohesion of society as a whole. To explain the concept of social exclusion scholars constructed a matrix of domains and topic areas looking across four stages of the life course: childhood, youth, working-age adulthood and later life. This matrix was called, the *Bristol Social Exclusion Matrix*, or B-SEM, contains three domains of potential importance in social exclusion under which there are ten sub-domains:²⁴

Table 4: The Bristol Social Exclusion Matrix

Three Main Domains	Sub-domains
Resources:	Material/economic resources
	Access to public and private services
	Social resources
Participation:	Economic participation
	Social participation
	Culture, education and skills
	Political and civic participation
Quality of life:	Health and well-being
	Living environment
	Crime, harm and criminalization

Understanding Vulnerability

Sources indicated that a number of social groups have been exposed to a certain kind of vulnerability. Based on the factors which trigger vulnerability, thus, it is important to define vulnerability.

²³ Khan, S., Combaz, E. & McAslan Fraser, E. (2015). Social exclusion: topic guide. Revised edition. Birmingham, UK: GSDRC, University of Birmingham.

²⁴ <https://www.poverty.ac.uk/definitions-poverty/social-exclusion> (Retrieved as of May 24, 2023)

Accordingly, the Pan American Health Organization (PAHO)²⁵ defined vulnerable populations as; “Groups that, due to biological, environmental or socio-cultural factors, have limited or no control over exposure to hazardous or risky situations.” Obviously, vulnerability affects the ability that young people have to successfully transition into adulthood. The transition can be affected by a number of situations: weak family and school ties; an insecure economic base; participation in risk-taking activities; and poor SRH decision-making.

Underlying Factors of Vulnerability

Poverty, gender and ethnic diversity were three factors aggravating poverty.

Poverty - is multi-dimensional, dynamic, and includes subjective dimensions such as powerlessness and voicelessness. It includes a lack of access to meeting basic needs such as food, shelter, clothing, sanitation, education, and health. The importance of income should be considered within the context of family and social support, inclusion in social, political, and economic institutions, access to other assets, and opportunities available. Gender, ethnicity, and age are important dimensions within poverty and should be seen as acting simultaneously with poverty. Poverty is also related to a lack of opportunity. SRH is related to poverty that lower household economic status is associated with poorer SRH indicators

Gender –masculinity and male attitudes towards sexuality and risk-taking behavior; alcohol and illegal drug use, the secrecy of same-sex activities, inconsistent condom use, and prestige associated with having multiple sexual partners, and attitudes towards interpersonal violence create vulnerabilities among certain groups of adolescent boys. Sexual violence and sexual coercion are often rooted in male notions of entitlement, control, and dominance. Among women and adolescent girls, cultural and social gender norms often restrict their access to basic information, prescribe an unequal and more passive role in sexual decision-making, undermine autonomy, and expose them to sexual coercion. Adolescent females are at an increased risk of early pregnancy, unsafe abortions, early motherhood, sexual exploitation, sexual abuse, and are less able to negotiate safe sex.

²⁵ Pan American Health Organization; Reaching Poor Adolescents in Situations of Vulnerability with Sexual and Reproductive Health. Washington, DC : PAHO, 2013. 525 Twenty-third Street, N.W., Washington, D.C. 20037, United States of America Tel.: +1 202.974.3000 Fax: +1 202.974.3663)

Ethnic diversity – studies revealed that the lack of information on the living conditions and health of minority ethnic groups is largely due to the fact that these populations are often invisible and overlooked. This deficit causes ethnic diverse populations to be excluded from national health plans, strategies and policies, and neglected by national health systems and their services. Other barriers preventing ethnic diverse populations to access health services include large geographical distances to services, inconvenient opening hours of health services, and the lack of cultural sensitivity among providers and health services. Ethnic background, together with low socio-economic status, rural residence, and lower educational levels are associated with lower contraceptive use, lower coverage of prenatal and delivery care, and shorter intervals between first and second births. Youth from minority groups/communities often face social and institutional discrimination impeding their access to health services that are due to: a) marginal political and legal status b) lower literacy levels and educational achievement c) barriers to economic opportunities and employment d) limited access and barriers to health and other services e) limited knowledge on SRH.

Study results from different sources and literature consistently reflects the situation of the GBIC. Harotessa Association-Ethiopia (HAE) were doing different projects to mitigate the problems resulted from social exclusion in different areas of their operational area. Projects were designed and implemented in target areas with similar problem. The organization was able to demonstrate commendable results in addressing social exclusion worth scaling up. Discrimination is Human Right violence. This violence is happened on the Gafat Bete Israel community for years. Women are often amongst the most vulnerable and the poorest of the community. Women have very limited access to and almost no control on the important economic resources.

6.3.2. Education and Vocational Skill Training

It is well agreed that education is a key tool to development. There is also an intrinsic relationship between education and income of poverty. Poverty and education are inextricably linked. Not every person without an education lives in extreme poverty, but most adults living in poverty missed out on a basic education. Poor people stop going to school because they have to work, which leaves them without the literacy and numeracy skills needed to improve their situation. With little income and few options, their children are also more likely to leave school, perpetuating a cycle of poverty that spans generations. Education directly correlates with many solutions to poverty: economic growth, reduced income inequality, reduced infant and maternal deaths, reduced stunting, reduced vulnerability to

HIV and AIDS, reduced violence at home and in society. For this reason, the fourth Sustainable Development Goal (SDG) of the UN 2030 Agenda relates to education: “ensure inclusive and equitable quality education and promote lifelong learning opportunities for all.”²⁶

Education is disseminated as the primary weapon against poverty incidence. By investing in people, education is a powerful driver of development and one of the strongest instruments for reducing poverty (World Bank, 2008). Furthermore, in April 2011, World Bank launched its Education Strategy 2020, Learning for All: Investing in People’s Knowledge and Skills to Promote Development, a road map for the next 10 years that focuses on increasing learning for all by investing early, investing smartly and investing for all. Learning for all means giving all people equitable opportunities to acquire the knowledge and skills they need to have healthy and satisfying lives, to be good citizens, and to be productive contributors to their countries ‘economic development’²⁷.

The relationship between education and poverty reduction is very clear: educated people have higher income earning potential and are better able to improve the quality of life. Education empowers; it helps people become more proactive, gain control over their lives and widen range of available choices. The combination of increased earning ability, political and social empowerment, and enhanced capacity to participate in community governance is a powerful instrument for helping break the poverty cycle. In fact, education is the primary vehicle by which economically and socially marginalized Filipinos can lift themselves out of poverty.²⁸

6.3.3. Adolescent, Youth Sexual and Reproductive Health (AYSRH)

By ways of the giving a perspective as regards to youth and young people the age aspect is defined as follows. Different countries define youth and young people in different ways. For example, for Uganda youth and young people mean those people in the age range between 12-30; Mauritius 14-29; South Africa 14-28; India 15-35; Nigeria 18-35; Djibouti 16-30. In this regard, Ethiopian Youth Policy defines youth and young people as people who fall in the age range of 15-29.²⁹ On the other hand, according to the African Youth Charter, youth and young people are those persons between the ages of 15 and 35 years. The African Youth Charter details a number of development issues directed towards youth of which health is one. The Charter in terms of health of youth and young people

²⁶ <https://mb.com.ph/2022/08/29/breaking-barriers-education-and-poverty/> (Retrieved as of May 24, 2023)

²⁷ Angeline M. Pogoy and Imelda C. Montalbo. CNU Journal of Higher Education 1 Special Edition of Poverty Alleviation, p1-10. An Empirical Evidence of the Impact of Education on Poverty Cebu Normal University. September 2012 (Retrieved as of May 24, 2023)

²⁸ Ibid

²⁹ FDRE, National Youth Policy. Ministry of Youth, Sports and Culture; March 2004. Addis Ababa

pointed out the need for identifying reproductive and health needs and designing responsive and relevant programs for vulnerable and disadvantaged youth. In addition it was asserted in the Charter that reproductive health services need to be youth friendly which includes variety of services such as contraceptive, ANC and PNC services. Introducing a comprehensive program geared towards preventing HIV/AIDS; STIs, harmful traditional practices including gender based violence and female genital mutilation (FGM) was also suggested with emphasis³⁰.

As indicated in the HSTP II, adolescents and youth constitute 42% of the total population in Ethiopia. However, about 20% of adolescent girls aged 15-19 years are anemic and underweight. Most young people in Ethiopia lack comprehensive knowledge on sexual and reproductive health (SRH), often engaging in risky behavior. Urban-rural disparities are wide as regards to comprehensive knowledge about HIV and HIV testing. About 37% of girls and 43% of boys aged 15-19 year consume alcohol. About 57% of boys also chew Khat. Despite high prevalence of health problems and risk factors among adolescents and youth, access to and utilization of health services is limited; and health education and life skills trainings for youth are often inadequate and fragmented (P.25).³¹

On the other hand, improving maternal health, promoting gender equality, and combating HIV/AIDS were considered to be three core issues and also identified as providing an entry point to address variables and indicators that effectively cross-cut the entire domain of reproductive health. Issues such as fertility, gender, age of first birth, contraceptive prevalence, method choice, traditional practices, literacy and other factors have direct implications for the health of men and women throughout their lifecycles. Responding to the socioeconomic and demographic realities of the country was the other priority. It was argued that although “inclusion” remains a hallmark of the concept of RH, the reality is that nearly 80 percent of Ethiopians live in rural areas; 60% are under the age of 20; nearly all are married by the age of 18; and few will ever reach their 55th birthday. So, it remains difficult to address the full range of activities theoretically subsumed under the rubric of reproductive and sexual health, SRH (PP 1&2)³²

The Reproductive Health Environment

RH being a comprehensive concept and implies a broad range of health and non-health related interventions, overview of the larger social and institutional contexts that influence RH in Ethiopia. It

³⁰ African Youth Charter. Adopted by the Seventh Ordinary Session of the Assembly held in Banjul, The Gambia; on 2nd July 2006

³¹ Health Sector Transformation Plan II (HSTP II) 2020/21-2024/25 (2013 EFY - 2017 EFY). February 2021

³² Federal Democratic Republic of Ethiopia MINISTRY OF HEALTH NATIONAL REPRODUCTIVE HEALTH STRATEGY 2006 - 2015

highlights the ways in which those conditions affect the RH status of women and men in the country. To this end, poverty, education, the legal environment, status of women and the health care system become crucial aspects. Poverty is considered as one of the most important determinants influencing the RH status. Ethiopia is one of the poorest countries in the world, with an estimated gross national product (GNP) per capita income of US\$100. Today, it is estimated that 47 percent of the population still lives below the poverty line (P.3).³³

Furthermore, investments in education, particularly for women, lead to better child health, lower fertility and reduced maternal mortality contributing to the success of the RH initiatives. However, educational status is a serious challenge in Ethiopia, where only 18.5 percent of women and 39.6 percent of men are literate. As far as legal environment is concerned while the Government of Ethiopia has adopted numerous laws and policies that advance women's social and reproductive rights, the implementation of these protections is constrained by low implementation capacity, low awareness among the general public and especially women (PP.3&4).

Status of Women

Status of women in Ethiopia is low. The low status of women in Ethiopia underpins, and often directly undermines each of the negative RH outcomes addressed in the national strategy. Most Ethiopian women lack the reproductive and social self-determination needed to exercise their reproductive rights – a condition that, in turn, perpetuates their low RH and social status. Despite a biological tendency towards higher survival rates, girl children are 4 percent more likely to die during infancy than boys. Between 60 and 80 percent of Ethiopian women have experienced some form of female genital cutting (FGC), a practice with potentially severe health consequences (P.4)³⁴.

Reports showed that the trend in controlling their earnings by husbands and wives is improving regardless of their educational background and urban-rural settings. However, slightly men have more dominance in rural areas than urban. Along with access to employment, control over cash earnings is a dimension of gender empowerment that women who are employed and who receive cash earnings are more likely to have control over their own earnings. In terms of ownership of assets, men's dominance is obviously high. Lack of assets makes women vulnerable to various forms of violence and affects her decision-making power in the family. Although the Ethiopian laws give

³³ Federal Democratic Republic of Ethiopia MINISTRY OF HEALTH NATIONAL REPRODUCTIVE HEALTH STRATEGY 2006 - 2015

³⁴ Federal Democratic Republic of Ethiopia MINISTRY OF HEALTH NATIONAL REPRODUCTIVE HEALTH STRATEGY 2006 - 2015

equal property rights to women, traditional misperceptions, attitudes and women's low social and economic status limits their ownership of assets. Women's ownership to land and house is unlikely low. This means that major assets are at the hands of men or their spouses which makes women weaker in power of negotiation.³⁵

Women's participation in decision-making is still low. It can be understood that women are considered to participate in decision making if they usually make that decision alone or jointly with their husbands. As well as the strength of the role of women in decision-making varies with the type of decision. In general, women's participation in decision-making increased with increased educational attainment. Besides, decision making power increases with education, urban women and women who are in the highest wealth quintile are more likely to participate in decision-making than rural women and those in lower wealth quintiles, women who are employed for cash are more likely than women not employed or not employed for cash to participate in decision-making, etc.

Gender-Based Violence

In Ethiopia, violence against women and girls continues to be a major challenge and a threat to women's empowerment. Women and girls face physical, emotional, and sexual abuses that undermine their health and ability to earn a living; disrupt their social systems and relationships; and rob them of their childhood and education. According to the EDHS 2016, among women age 15-49, 23% of them have experienced physical violence and 10% have experienced sexual violence. Four percent of women have experienced physical violence during pregnancy.

In terms of marital control, 16% of ever-married women have experienced at least three types of marital control behaviors by their husbands or partners. Concerning spousal violence 34% of ever-married women age 15-49 have experienced spousal physical, sexual, or emotional violence. Physical and emotional violence were experienced by 24% each and sexual violence by 10%. Injuries due to spousal violence were also reported. According to the report 22% of ever-married women who experienced spousal physical or sexual violence reported injuries, including 19% who reported cuts, bruises, or aches and 10% reported deep wounds and other serious injuries.

A study conducted by UN Women indicated that 25% of Ethiopian women experienced their first sexual experiences under coercion. The study also showed a high level of acceptance of violence against women, both by women and men by asserting that over 69% of the respondents beating a

³⁵ Ethiopian Demographic and Health Survey, 2016

wife is justifiable for reasons such as refusing sex, neglecting a child and arguing with a husband. The study also pin pointed that 10% of married young women aged 15 to 24 years, experience physical domestic violence and 7% of ever-married women had experienced psychological abuse and insults. Though there is some statistics on early marriage, FGM and to some extent on reported cases of civil and crime cases, nonetheless, other forms of violence such as economic and psychological violence may be underreported or even not recognized, according to the study.

Studies indicated that absence of Civil Remedies for Victims or Survivors remains serious gap. It is noticed that the Ethiopian law lacks any kind of civil remedies against domestic violence. It was identified that the law does not have separate domestic violence act or law which provides specific civil remedies for victims/survivors such as right to obtain protection order, monetary or compensation relief, custody order, residence order, shelter or medical benefits or more than one such order. Secondly, the law does not have sufficient Criminal Liabilities on Perpetrators. It is obvious that there are some affirmative improvements, but the new Ethiopian Criminal Code has still some critical gap of addressing the problem of domestic violence in sufficient manner.³⁶

6.3.4. Enhancing Livelihood Opportunities for youth and women

Household Poverty and Unemployment

Household poverty is affecting marginalized communities. Household poverty is a cause of poor health and education outcomes, and is believed to be one of the key drivers for the spread of HIV/AIDS. Women are among those groups of communities that are being affected most and are forced to be engaged in commercial and transactional sex and are exposed to exploitations and abuses. Unemployment and underemployment especially in younger people (15-29 years old) is continuing to be a serious challenge, including for those who have finished secondary education, TVET and college graduates.

The recent urban employment and unemployment survey indicates high youth unemployment of 23% (29.6% female and 16.4% male), which indicates the problem is even more severe for women. The problem partially imparts on lack of access and quality skills training beyond secondary education – that focuses on equipping youth with employable skills or that opens market opportunities to create job, addressing the tendency to expect work that will pay them a salary, and a lack of innovation and initiative to become self-employed. Youth and women have been exposed to illegal migration and are

³⁶ Megersa Dugasa Fite. The Ethiopia's Legal Framework on Domestic Violence against Women: a Critical Perspective. *International Journal of Gender and Women's Studies*. March 2014, Vol. 2, No. 1, pp. 49-60
ISSN: 2333-6021 (Print), 2333-603X (Online)

in increased susceptibility to other socio-economic and health risks, in addition to keeping them in the cycle of poverty.

Apart from lack of access to information related to employment, they have no necessary skills and knowledge that can help them engage in safer livelihood opportunities and improve incomes to support their lives and the lives of people who live with them. To this end, interventions in livelihood development for women and youth and those that create opportunities to enhance their skills and improve income are necessary. The objective of the Livelihood Program is, hence, to reduce poverty and inequality by generating employment among poor households and by moving highly vulnerable households into sustainable livelihoods and toward economic stability. Thus, livelihood program focuses on the following major areas.

These are providing skills trainings for economic empowerment; create jobs and employment opportunities for vulnerable youth and women; facilitating access to finance, trainings, market and linkages for micro and small enterprise development; collaborating with TVET agencies in providing formal and non-formal skills training for the youth to create employment or business opportunities; and promoting opportunities for job creation, entrepreneurship and linkages with Micro and Small Enterprises

6.4. Target groups and beneficiaries

Across the interventions, HAE thoroughly considers who the primary beneficiaries and end users are and would be. HAE believes that its very existence as an entity is to serve project beneficiaries and target groups. In this regard, HAE is concerned over positively shaping life of the beneficiaries through its interventions. Thus, HAE have a philosophy in terms of selection, targeting and reaching people and working with people. HAE identified and is going to identify unique beneficiaries and target groups to reach and work together under each intervention.

Instead, HAE targets, selects and reaches beneficiaries systematically using selection criteria that avoid bias and which is evidence and right based. HAE during the project design considers vulnerability, demographic and gender parameters to elucidate that its programming are right-based; gender sensitive and addresses vulnerability and critical development issues. In this regard, programming of HAE considers the following target groups and beneficiaries.

The target beneficiaries will be marginalized Gafat Beta-Israel community specifically adult women and young women who are neglected and socially excluded. These people are characterized by their poor living condition, poor sanitation, and substandard shelter. The target communities living in the

target areas are the poorest of the poor and socially marginalized where traditional pottery workers do live. Most of the unemployed beneficiaries are engaged in different informal and disorganized income generating activities and start making money to sustain their life. In addition, the beneficiaries lack economic capacity to send their children to school and thus, majority of their school age children are not participating in formal primary education.

7. CROSS-CUTTING ISSUES

7.1. Promoting gender equality and empower women

Considering the gaps that exists in access to services and participation for men, women, boys and girls, HAE will conduct a robust gender analysis as part of planning, and also incorporate strategies to increase women and girls' participation and representation in the implementation, monitoring and reporting, to make sure that needs and concerns both genders to clearly identify and respond. HAE's commitment for gender equality will also be visible at organizational level through paving the paths for increased representation of women at different roles, including the board and key staff.

7.2. People with Disability (PwD)

HAE strongly believes in the inclusion of the marginalized and underserved people and groups. In this regard, the organization ensures inclusion of the PwDs in its project cycle management (design, implementation, monitoring and evaluation) and committed towards ensuring meaningful inclusion and effective participation of the PwD in program/project processes and decisions. The organization will not discriminate people based on disability status in any form, in its programs, services and staffing. Monitoring tools will be customized to capture data on PwDs as well.

7.3. Promoting Environmental Friendly Technologies and Approaches:

HAE cares for the environment in order to minimize the challenges that the world is facing due to climate change. The organization practices environmental friendliness in its work and mainstreams this across the range of project portfolios.

7.4. Child Protection and Safeguarding

Understanding the risks of harm to children that can arise from a range of sources –from within the family and community to their participation in work or school and related programs or through accessing services; HAE will put in place everything possible to prevent harm arising on children involved in programs, respond to harms once they appeared and build reporting and service linkages with relevant service providers. Child protection policy with the code of conduct will be developed and everyone working for or with us will sign and acknowledge the acceptance in advance.

7.5. Conflict Resolution and Management

HAE recognized frequent occurrence of conflicts and the resultant loss of life, property and trauma. HAE also believes that conflict erodes the value system of the society and impedes development undertakings. To this end, HAE aspires to be engaged in conflict resolution and management endeavors. The organization cultivates indigenous conflict resolutions and management approaches. Strengthening early warning systems may be some of the methods HAE will consider.

8. STRATEGIC FRAMEWORK AND OBJECTIVES

8.1. Strategic Framework



Figure 6: Strategic Framework

8.2. Strategic Objectives (SO) and Proposed Actions

SO1: Social Inclusion and Gender Equality

Key intervention areas under this strategic objective:

- Assess and map the social and economic situation of the GBIs and **ultra-poor** section of the communities in the project targeted area.
- Increase GBI communities' educational participation and access to health services
- Enhance community awareness on gender equality and social inclusions.
- Enhance HAE and sector offices instructional capacity for gender mainstreaming and gender responsive and social inclusion service provisions.

SO2: Education and Vocational Skill Training

Key intervention areas under this strategic objective:

- Establish four Vocational & Training centers for modern pottery production and construct shades at Hadiya; Kambat & Tambaro; Gurage and Dawro zones
- Provide different kinds of vocational skill training on modern pottery production
- Establish anti marginalization club in schools and provide orientation training on awareness creation on Gafat Beta Israel communities' education challenges
- Enhance equitable education service delivery with focus on marginalized children and adult Gafat Beta Israel community
- Establish and facilitate school libraries, establish and equip pedagogical centers and strengthen ICT infrastructure

SO3: Adolescent, Youth Sexual and Reproductive Health (AYSRH)

Key intervention areas under this strategic objective:

- increase awareness and utilization of quality SRH and SGBV services by the vulnerable groups improve accessibility and availability of integrated package of SRH services
- strengthen institutions and social environment to enable them effectively respond to the rights and needs of vulnerable groups
- improve networking and partnership to strengthen collaboration and engagement of stakeholders through

- Promote family planning education and information dissemination, increase contraception use and reduce unintended pregnancy for adolescent youth
- Promote youth friendly services within public service providers and ensure youth friendliness in the healthcare services that HAE is providing
- Promoting and engaging in safe space program for girls which believed to increase knowledge of girls on SRH issues and minimize vulnerability to different hazards/risks

SO4: Enhancing Livelihood Opportunities for youth and women

- Provide skills trainings for economic empowerment, create jobs and employment opportunities for vulnerable youth and women
- Facilitate access to finance, trainings, market and linkages for micro and small enterprise development
- Collaborate with TVET agencies in providing formal and non-formal skills training for the youth to create employment or business opportunities
- Promote opportunities for job creation, entrepreneurship and linkages with Micro and Small Enterprises

9. FINANCIAL AND HUMAN RESOURCE PLANNING AND MANAGEMENT

HAE understands the critical needs in strengthening and sustaining resources that are keys for achieving its mission. Hence it has aligned the financial and human resources plan during its five-year strategic plan, as presented below.

9.1. Budget Planning and Financial Resources Management

Three years' income (2022-2023) of HAE preceding this strategic plan has been reviewed. Trend of income showed a steady decline in the last two years (i.e. 2022 and 2023). In 2023 HAE's operating was in the range of 985,509 (Birr Nine hundred eighty five thousand five hundred nine). Compared to the 2021 annual organizational income which was birr 1,036,503, there is slight difference in terms of growth of revenue. But, the income of the association couldn't maintain the 2021 budget or didn't show significant growth.

Taking the above scenario as a reference and taking average of the past three years' association's income (i.e. Birr **1,001,220**) as a benchmark, HAE's five years' income is forecasted. To this effect, HAE needs to strengthen its internal capacity to mobilize funds and generate income in order to uplift its budget by 5% in the first year (2023); 10% in the second year (2024) and by 15% (2025-2027) in each of the remaining three years during this strategic plan period and reach at least birr 1,151,403 for the final year (5th year) of this strategic plan. The total estimated budget for this strategic plan is birr **5,606,832 (Birr five million six hundred six thousand eight hundred thirty two)**, subject to inflation (See Table 4 for details). This will help the organization achieve its planned targets and put itself in a better shape towards achieving its mission. This will also set a fine start to the next strategic period, improving the organization's budget more than 2 times after five years.

The HAE will strive to achieve financial sustainability by implementing three fundamental activities. One is diversifying budget/income sources which guarantees wider resources base and puts the organization in strong position by avoiding risks associated with financial and budget shortages. The second is revamping membership schemes and boosting membership contributions. Finally, HAE may introduce innovative income generating and fund raising schemes. The program team and management may play a pivotal role in fundraising and planning for a robust resource mobilization by taking the advantage of the situation of the community and the SDG agenda.

Table 5: HAE's Income History and Five Years' Financial Resource Forecast (2024-2028)

I. HAE's Income History			
S/No	Year	Organization's Income for the Past three consecutive years (Birr)	Remarks
1	2021	1,036,503	
2	2022	981,647	
3	2023	985,509	
Total		3,003,659	
Average (2020-2022)		1,001,220	
II. Financial Resource Forecast (2024-2028)			
S/No	Year	Forecasted/Projected Amount for the SP Period (Birr)	Remarks
1	2024	1,051,281	Projected at 5% increment
2	2025	1,101,342	Projected at 10% increment
3	2026	1,151,403	Projected at 15% increment
4	2027	1,151,403	
5	2028	1,151,403	
Total		5,606,832	

9.2. Organizational Setup and Management

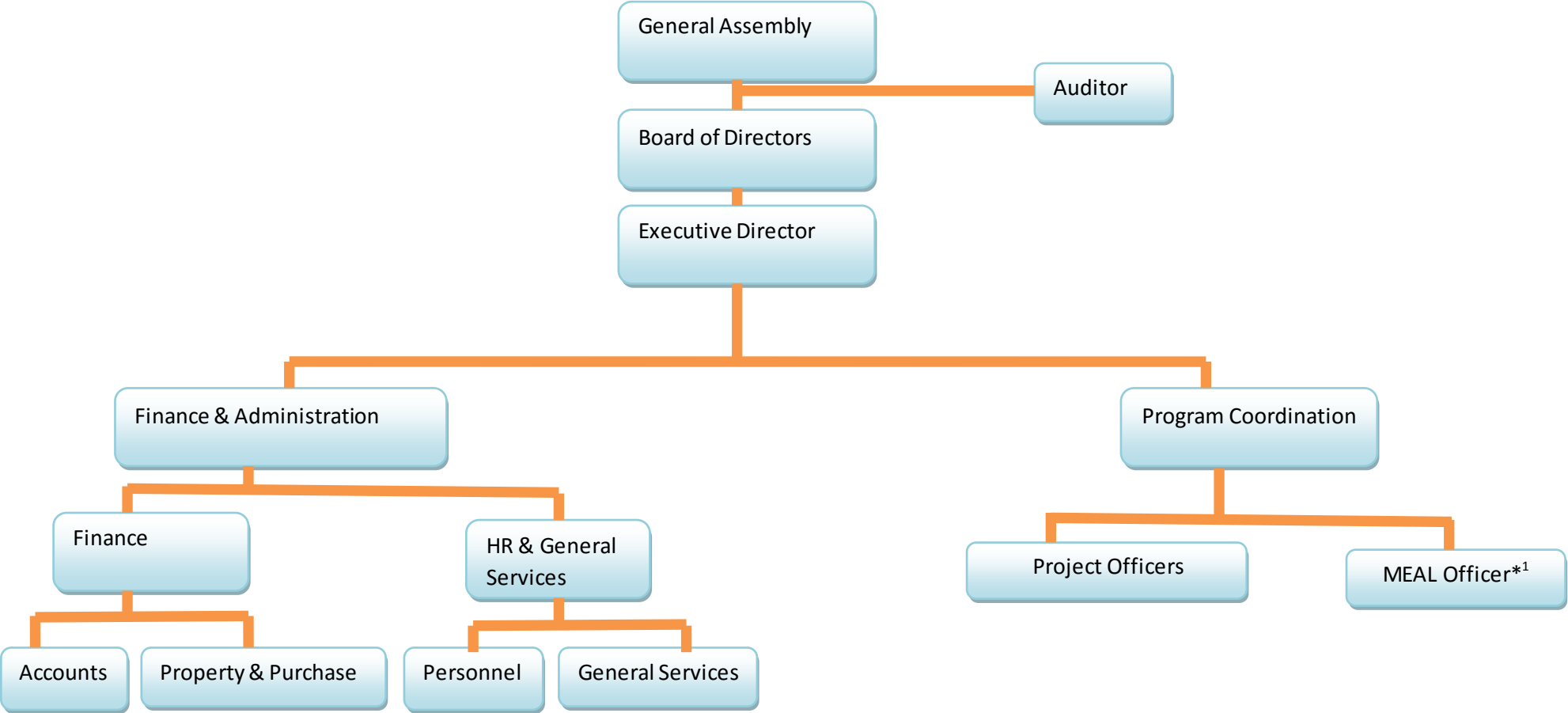
The current offices and organizational leadership remains during the implementation of this strategic plan. As per the organizational bylaw approved by the ChSA, the boards of directors, backed by the general assembly, and with the assistance of the Executive Director, will execute strategic actions supportive to the organizations technical and operational effectiveness. As illustrated in the organogram annexed to this document, HAE clearly reflected the organizational setup and management such that the highest constituency is the general assembly –which elects the board.

The organization's board oversees the organization's operation and takes key decisions –including approving the annual reports and plans. The Executive Director, as a fulltime designate, and also serving the board as a secretary, manages overall organizational efficiency and effectiveness human resources and budget, while the program coordinator also take the direct and primary leadership roles in program management, organizational development and resource mobilization, partnerships and

networking. Similarly, the Finance and Administration officer leads the overall financial and administration issues of the association.

In line with the need, the organization will review the current staffing and staff roles, reporting lines and recruit and fill more positions. When expansion to other geographical locations becomes evident, pursuant to the availability of funds and needs, Branch Offices and other essential staff will be accordingly deployed. In addition to this, depending on the nature of the projects and needs of the donors, professionals in specific professions may be needed.

9.3. Organizational Structure



*1=Monitoring, Evaluation, Accountancy and Learning

Figure 7: HAE's Organizational Structure

10. MEASUREMENT OF SUCCESS/MONITORING PLAN

10.1. Measurement of Success

The aim is to specify clearly how the organization will measure the success of its activities. This will be a measurement of its services; details of what to measure and how to measure the various achievements of the organization is explained; what to measure will generally be based on goals and tasks set by the organization to be achieved during the plan period. How to measure success will be a methodology used to measure the actual performance of the organization compared to the targets set for each goal. In this regard, the organization revises and updates its existing monitoring and evaluation guideline to fit in to the current SP and indicators set in the SP.

10.2. Financial Projections

The strategic plan will be quantified to monitor performance and evaluate the results achieved; draw up revenues and expenses for the current period and the plan period; for the immediate current period, the annual plan is broken down into monthly or quarterly plan for effective monitoring of actual performance and timely correction of deviations; while KPIs measure the efficiency of individual activities that constitute an organization's overall operations, Financial Projections (targets) monitors the overall health of the organization.

In non-for-profit organizations where profit is not the motive, financial projections fix expense budgets to be within allocated resources for various activities; an organization can spend only what it raises as grants and funds from its beneficiaries and hence everyone in the organization should know how much they can spend on various activities to be within the available funds' position.

Where expenses go beyond the projected limits, the organization management takes corrective action to bring down such expenses; an organization financial projection also forecasts grants to be received and funds to be raised during the plan period. The staff team entrusted with this activity should ensure that the targeted grants and funds are raised to meet the expenses planned during the plan period. Where it is found that the planned fund-raising is falling below targeted values, the organization management should increase its efforts in this regard to raise the shortfall grants and funds.

Mulatu Kacha Arato

Harotessa Association Ethiopia G /A President

December 30 / 2023 G.C

